Beginnings Fund Strategic and Technical Plan (Fund One: 2024-2030)

Investment Overview

Geographic Focus:	Ethiopia, Ghana, Kenya, Lesotho, Malawi, Nigeria, Rwanda, Tanzania, Uganda, Zimbabwe. Other African countries will be considered during Fund Two.
Fund One Target Amount:	USD525million
Fund One Term:	76 months ending 31 December 2030
Funds Raised to Date:	USD492.678million (Children's Investment Fund Foundation, Delta Philanthropies, The ELMA Foundation, the Gates Foundation, Mohamed bin Zayed Foundation for Humanity, private donors, and Anonymous)
Description:	The Beginnings Fund aims to accelerate maternal and newborn survival in Africa by pooling and deploying philanthropic capital to support governments to scale high-impact interventions targeted at the primary drivers of mortality during pregnancy, labor and delivery, and the newborn period. During Fund One (1 September 2024 – 31 December 2030), Beginnings Fund will invest in up to 10 African countries, in partnership with governments, to improve the quality of maternal and newborn care in high-burden district hospitals and their referral networks by: • Increasing the availability, quality and uptake of key innovations and products • Strengthening and expanding the health workforce • Investing in referral and emergency transport systems • Supporting robust data systems to enable quality improvement and decision-making The Beginnings Fund will advocate for financing, prioritization, and uptake of this approach beyond target geographies.
Measures of Success:	Targeted Impact: Over 322,000 lives saved during 2025-2030, with sustained reductions in national newborn mortality, maternal mortality, and stillborn rates. See Annex A for the full list of impact and outcome indicators.

Acronyms and Definitions	
ANC	Antenatal Care
EmONC	15-30% of pregnant women and newborns will experience complications requiring emergency obstetric and newborn care (EmONC) in health facilities where key functions are performed regularly to ensure high quality. Many EmONC-designated facilities are not providing these functions due to gaps in products, infrastructure, and workforce
ENAP/EPMM	Every Newborn Action Plan (ENAP) and Ending Preventable Maternal Mortality (EPMM) are global initiatives led by WHO, UNICEF, and UNFPA which support countries aiming to end preventable newborn and maternal mortality and stillbirths. One key goal is 80% of districts having access to EmONC and small and sick newborn care. All targeted Beginnings Fund countries have ENAP/EPMM acceleration plans which identify priorities for government and donor alignment.
iKMC	Immediate Kangaroo Mother Care (iKMC) is prolonged skin-to-skin contact from birth for all babies weighing <2.5kg, for improved sepsis prevention, breathing, feeding, and temperature control.
MNH	Maternal Newborn Health
MOH	Ministry of Health
SSNC	Small and Sick Newborn Care

Overview and Strategic Approach

Every woman deserves a healthy pregnancy and every newborn, a healthy start to life. Yet mothers and newborns in Africa struggle to survive preventable pregnancy-, birth-, and postpartum-related complications. Every year, 1.1 million newborns die,[1] 847,000 babies are still born,[2] and 202,000 women die[3] from preventable causes related to pregnancy and childbirth in Africa. Neonatal conditions are the leading causes of death in Africa.[4] The major causes of maternal and neonatal deaths and stillbirths are known, but the diagnostics, medicines, equipment, and interventions that save lives are not reaching the mothers and babies that need them. Through collective philanthropic action, there is an opportunity to accelerate maternal and newborn survival in Africa, beyond what any donor can tackle individually. The Children's Investment Fund Foundation (CIFF), Delta Philanthropies (Delta), The ELMA Foundation (ELMA), the Gates Foundation (Gates), and the Mohamed bin Zayed Foundation for Humanity (MBZFH) are founding and anchoring the Beginnings Fund and will collectively mobilize funding from other philanthropies to reach an initial Fund One target of USD525million to support proven, high-impact interventions to drive down maternal and newborn mortality and stillbirths. Fund One will be deployed over 76 months (1 September 2024 – to December 2030) in partnership with governments in Ethiopia, Ghana, Kenya, Lesotho, Malawi, Nigeria, Rwanda, Tanzania, Uganda, and Zimbabwe.

Most births in Beginnings Fund countries take place in health facilities (>80%) where the quality of care for women and newborns is currently lacking, resulting in preventable morbidity and mortality. Beginnings Fund investments will target the largest drivers of mortality during the period when most deaths occur – labor, delivery, and the newborn period. Beginnings Fund will invest in districts with the highest burden and their hospital referral networks, led by government and with consortia of implementing partners, to deploy high-quality **innovations and products**, strengthen and expand the **health workforce**, and enable **critical health systems** – emergency transport and referral, and data for decision-making and quality improvement. By investing in districts with 50% of the institutional maternal and neonatal deaths and stillbirths in each country, this initiative has the potential to approximately 322,000 lives by 2030 and sustain reductions in these mortality rates into the future, saving many more lives for years to come.

Current efforts to address maternal and newborn mortality and stillbirths are fragmented, often lacking alignment with government plans and a shared vision of success. This is compounded by a lack of timely and sufficient funding. Most initiatives focus on a single or limited set of interventions, do not address the fundamental gaps in workforce numbers or specialist training, and do not support sustained access to quality devices, medicines, and supplies. Beginnings Fund aims to shift the way in which maternal and newborn investments are made, intentionally combining investments across products, workforce, and systems to enable quality care. Beginnings Fund will disrupt the status quo and bring greater attention to ending preventable perinatal deaths by rapidly improving staff:patient ratios, and equipping health facilities with high-quality products, renovations needed to enable quality care, and requisite data and referral systems. Governments and donors are highly focused on improving access to primary health care, which is essential, but the gap in ensuring a sufficient, well-trained, and equipped health workforce at hospitals and their referral facilities is evident. As such, this is where Beginnings Fund will prioritize its initial efforts.

To ensure the political commitment necessary for adoption and sustained scale, Beginnings Fund will negotiate financial and in-kind commitments upfront with ministries of health and finance. As a philanthropic collaborative, the Beginnings Fund is well-positioned to be agile and flexible, to pivot implementation, take on new innovations and effective approaches, and respond quickly to emerging government priorities. Beginnings Fund will **invest** in the implementation of proven solutions where they will have the greatest impact; **complement** existing government and bilateral/multilateral investments for greater reach; and **influence** scale-up through financing and policy advocacy to

[1] (2024) UN IGME Levels & trends in Child Mortality, 2023 Report.
[2] (2022) UN IGME Stillbirth Report. Never Forgotten – the situation of stillbirth around the globe.
[3] (2023) MMEIG Trends in maternal mortality 2000 to 2020.
[4] WHO AFRO Fact Sheet June 2023, WHO Global Health Estimates (latest release 2019)

ensure uptake of the combined interventions beyond the target geographies. Should Beginnings Fund and its partners achieve early indications of results, Beginnings Fund will raise a second round of funding (Fund Two) to achieve the ten-year goal of deploying USD1billion towards high-impact maternal and newborn survival programs in at least ten African countries.

Beginnings Fund will focus on sustained impact from inception, by co-designing country plans with government and national health experts, and strengthening the capacity of national organizations. This will include improving product regulation, procurement and supply chain management to enable sustained access to higher-quality products at the same or less spend, and efficient integration and uptake of product innovations. It will also mean strengthening the quality of pre-service and post-graduate training – including teaching hospital environments – to enable successive cohorts of trainees to learn in best practice settings while supporting health workforce planning and regulation for improved production, absorption, and retention of priority cadres. Beginnings Fund will support country plans to expand specialist workforce training through fellowship models to reduce the cost of training, improve care at training sites, and exponentially grow the specialized workforce. Strengthening national pediatric, obstetric and midwifery associations will enable responsive clinical mentoring and will support perinatal death audits and actions to address preventable factors. Increased capacity to monitor coverage of key interventions will be supported through integration of inpatient maternal and neonatal data within national health information systems at the outset. Strengthening the management processes and data visualization at the national and sub-national levels so that they can engage with and respond to this information will help strengthen care beyond the lifetime of this fund. Support for setting-up national emergency transport and referral systems, in partnership with other donors, will enable governments to absorb the modest operating costs thereafter.

The fund will adopt a lean operating structure and leverage the diverse expertise from founding donors. **CIFF** has deep experience co-investing with African governments and supporting implementing partners to address maternal and newborn health (MNH) in Kenya, Ethiopia, and Ghana, combined with evidence generation and robust data analysis. **Delta**, through its implementing organization, **the Higherlife Foundation**, partners with the governments of Zimbabwe and Lesotho on human capital development initiatives, including MNH services; its deep connections with communities and stakeholders ensure an in-depth understanding of the local context and the challenges they seek to address. **Gates** has significant experience working closely with governments and key industry, private sector, academia, research, and civil society partners to scale up high-impact products and innovations addressing the key drivers of maternal, newborn and child mortality and morbidity in Kenya, Ethiopia, Nigeria, Rwanda, and South Africa. **ELMA** has experience supporting workforce investments with consortia of partners and aligning those investments with efforts to improve maternal and newborn service delivery in Ethiopia, Kenya, Malawi, Rwanda, Tanzania, Uganda, Zambia, and Zimbabwe. The **Mohamed bin Zayed Foundation for Humanity** has experience through establishing the Reaching the Last Mile alongside Gates to eliminate polio, neglected tropical diseases, and malaria, in over 45 countries in Africa, Asia, and Middle East.

The Beginnings Fund will build on the founding partners' existing maternal and newborn health investments, and complementary investments in medical oxygen, sexual and reproductive health and well-being of adolescent girls, and community health systems. It will also leverage the founding partners' relationships with key leaders in the field, their networks of strong implementing partners, and staff expertise from their offices in Abuja, Addis Ababa, Cape Town, Harare, Kampala, Johannesburg, and Nairobi.

Situation Analysis

Every year, 1.1 million newborns die,[3] 847,000 babies are still born,[4] and 202,000 women die[5] from preventable causes related to pregnancy and childbirth in Africa. Three maternal complications: postpartum haemorrhage, sepsis, and eclampsia account for half of all maternal deaths. Pre-term birth complications, asphyxia, and newborn infections

account for almost 80% of newborn deaths. Most stillbirths are caused by intrauterine hypoxia, often preceded by clinical indications of preeclampsia, placental abruption, and small-for-gestational-age growth restriction.

Beginnings Fund countries are making the major shift to having most births take place in health facilities. Central to the Beginnings Fund's approach is maximizing the quality of antenatal, intrapartum, and small and sick newborn care delivered in high-volume hospitals and their referral networks, where most deaths occur. For example, in Kenya 60-70% of deaths occur at county and sub-county referral hospitals.[5] In Rwanda, 78% of deaths take place at district hospitals.[6] Ideally, all births would take place in a health facility where mothers and newborns receive high quality care. In practice, these facilities are poorly staffed, with the health workforce often lacking the necessary skills, equipment, products, and information to deliver quality care. High-impact innovations are either non-existent or not deployed at sufficient scale. Since 15-30% of all pregnant women and newborns will experience complications requiring comprehensive emergency obstetric and newborn care (CEmONC) or small and sick newborn care (SSNC), hospitals designated for this must be accessible and have sufficient health workers equipped with skills, quality medicines, and tools. To accelerate progress, health systems need to evolve to ensure intrapartum monitoring, rapid access to caesarean sections when needed, and specialized newborn care for all births that take place in facilities.

Limited **human resource capacity**, in terms of numbers and the right skills-mix, is a key barrier to quality care. There are major shortages in both specialist physicians such as obstetricians/gynecologists and pediatricians, and specialist nurses, particularly midwives and neonatal nurses. All the Beginnings Fund countries fall below the WHO health worker density target of 4.5 health workers per 1000 population for achievement of universal health coverage (UHC). In Kenya and Zimbabwe, the density for doctors, nurses, and midwives ranges between 1.0 to 2.2 per 1000 while all the other countries have a density of <1.0 per 1000.[7] Estimates from labor market analyses indicate that unemployment for nurses and midwives in many African countries ranges between 17% and 37% within one year of their graduation,[8] largely due to limited fiscal space in the public sector. In a recent study, the median staff:patient ratio for nurses in neonatal units in Kenya's public sector was 1:19 (global standards recommend a ratio of 1 nurse to 1 baby, or a maximum of 4 babies in stable environments), with babies receiving only 42% of the expected nurse-delivered care.[9] Emerging data (unpublished) from seven hospitals in Kenya demonstrates that a decrease in nurse staffing ratio from greater than 1:24 to 1:18 resulted in a 22% reduction in mortality after accounting for disease severity.

SRMNH Workforce[1]	Global Average	Ethiopia	Kenya	Lesotho	Malawi	Rwanda	Tanzania	Uganda	Zimbabwe
SRMNH Workforce density per 10,0000 population (2020)	47.3	7.3	27.9	57.6	15.3	10.3	7.3	14.1	31.1
# of midwives needed (2020)		62,300	8,200	not available	6,400	6,900	25,200	26,100	not available

1) The State of the World's Midwifery 2022, East and Southern Africa

Most nurses in neonatal units do not have formal training, with only a handful of neonatal nurse training programs on the African continent. Universities and colleges training maternal and newborn health cadres need support to transition from classroom-based to competency-based approaches, utilizing skills labs and simulation to generate the many

[5] Kenya Health Indicator Survey
[6] Confidential Enquiry into Perinatal Deaths in Health Facilities, 2020
[7] Ahmat A, Okoroafor SC, Kazanga I, et al The health workforce status in the WHO African Region: findings of a cross-sectional study BMJ Global Health 2022;7:e008317.
[8] WHO. State of the world's nursing 2020: investing in education, jobs and leadership. 2020.
[9] Gathara D, Serem G, Murphy GAV, Obengo A, Tallam E, Jackson D, Brownie S, English M. Missed nursing care in newborn units: a cross-sectional direct observational study. BMJ Qual Saf. 2020 Jan;29(1):19-30.

emergency scenarios graduates will face. Greater focus on interdisciplinary team approaches is needed in learning and in practice as effective emergency obstetric and newborn care and care for small and sick newborns require teamwork and communication across maternal, neonatal, anesthesia, and biomedical cadres.

Availability of high-quality and affordable **products,** including medicines, diagnostics, and devices, is critical for health workers to provide quality care. In targeted countries, essential MNH products are of low quality and only available in 45% of facilities, due to low volumes and high prices. Median prices for drugs are 2.7 times higher in the public sector and 6.3 times higher in the private sector than the global reference price. This substantial variation is in part due to a lack of uniformity and regulatory standards in procurement practices. Kano State in Nigeria, for example, pays USD0.63 for oxytocin (triple the global price), and Kaduna State pays 20 times more than Uganda for misoprostol. Even when products are purchased in sufficient volumes with affordable pricing, the quality is not always assured. In a recent WHO survey, uterotonic quality failures range from 43-78%, translating to wastage and overuse. A study in Northern Nigeria in 2021 found providers typically used 3 to 4 times the standard dose of oxytocin to achieve the same effectiveness. Emerging products, such as azithromycin, low-dose antenatal corticosteroids, and point-of-care ultrasound, are not updated in government guidelines and their use is lesser known to providers, resulting in significantly lower volumes procured, thus not meeting the minimum order quantities of higher quality suppliers. Further government MNH product spend, which amounted to USD50million across Ethiopia, Kenya, Nigeria, Tanzania, and Uganda in 2020, is highly fragmented. The 14 products procured included 110 brands from 67 manufacturers.

An overall lack of the device and supply bundle needed for small and sick newborn care is prevalent, with hospitals poorly equipped to prevent and treat respiratory distress with CPAP, identify and treat jaundice and infections, keep babies warm and fed, and provide KMC, especially for unstable babies requiring additional interventions. Many hospitals do not yet provide small and sick newborn care, leading to delays in vulnerable newborns accessing timely care or preventing access to care altogether.

Other **health system** gaps inhibit quality maternal and newborn care. National MNH indicators only provide aggregate data which do not reflect the quality of care provided across the antenatal, intrapartum, and newborn period. While WHO is close to finalizing a set of MNH quality-of-care indicators to monitor effective coverage of interventions, countries will need support to build the patient-level data systems to generate this information. Gaps in emergency readiness, stabilization and referral practices, and shortage of functional ambulances cause delays, especially for intra-facility transportation during obstetric emergencies. Once women make it to the referral hospital, emergency obstetric care is often further delayed due to poor communication resulting in occupied operating theatres. Many women also deliver, recover, and care for small and sick newborns in severely sub-optimal settings, without access to adequate furniture, water and sanitation facilities, and well-equipped maternity units. To deliver effective interventions such as early detection and bundled management of PPH, providers require delivery beds that support detection drapes, stocked and uniform emergency trolleys, and other critical supplies which are missing in many facilities.

Beginnings Fund will make transformative investments across evidence-based maternal and newborn health innovations and products, health workforce, and critical systems necessary to drive and sustain reductions in mortality.

Governance and Organizational Structure

The Beginnings Fund is governed by an Investment Committee and managed by an Investment Team that develops high-impact investments with support from a Technical Advisory Group. Beginnings Fund is fiscally sponsored by Global Impact, which receives, manages, and disburses donor funding, based on grant recommendations developed by the Investment Team and approved by the Investment Committee.

Investment Committee (IC): Comprised of a representative from each founding donor, and new donors committing USD50million or more to Beginnings Fund, the IC provides strategic and management oversight of the fund, and approval of new donors. The founding IC members are Faustina Fynn-Nyame (CIFF); Rasa Izadnegahdar (Gates); Robyn Calder (ELMA); Elizabeth Tanya Masiyiwa (Delta), and Tala Al Ramahi (Mohamed bin Zayed Foundation for Humanity). The committee meets at least quarterly to review and approve budgets, country plans, and investment recommendations presented by the Investment Team. The IC will hold an annual strategy review meeting with the CEO and the Investment Team to analyze key successes and challenges, discuss course correction strategies, and approve revised work plans and budgets.

Investment Team: Under the guidance of the IC and with support from the Technical Advisory Group, this team develops high-quality investment recommendations that meet the fund's strategic objectives and effectively manages approved investments. The team structure is lean, resourced by staff from the core donor organizations as well as direct hires. The founding team is composed of a CEO, two Program Directors, a Director of Statistics and Impact Evaluation, a Program Measurement and Evaluation Manager, a Senior Program Officer, a Director of Finance and Operations, and an Executive Assistant.

Reporting to the Investment Committee, the CEO provides overall leadership in developing and executing the Beginnings Fund strategy, oversees all program activities and operations, and leads advocacy and fundraising. The Program Directors are responsible for strategic technical leadership and coordination, country investment plan development, developing and recommending grants, monitoring those grants, and working closely with one coordinating implementing partner per country. The Director of Statistics and Impact Evaluation leads the fund's measurement, evaluation, learning, and evidence building. Working closely with the Program Directors, the Senior Program Officer manages investment planning, development, and monitoring, and provides support on measurement, evaluation, learning, and operations where needed. To maintain the lean structure, short-term consultants may be utilized to support grant development. The Director of Finance leads grants finance and oversees finance operations. The Executive Assistant supports daily operational processes, communication, and grants administration. The investment team draws on the deep bench of expertise from the founding partners on a regular basis.

Technical Advisory Group (TAG): This group comprises technical and clinical experts with experience in product introduction, health workforce, quality improvement, and government relations, from core donor organizations that serve as advisors to the fund. The TAG reviews country plans and investment work plans before approval by the IC. Any additions to the Technical Advisory Group are subject to approval by the Investment Committee. Like the Investment Team and Investment Committee, the TAG will remain lean and adopt a hands-on approach over the life of Beginnings Fund.

See Annex B for members of the Investment Team, Investment Committee, and Technical Advisory Group.

Fiscal Sponsor: Global Impact is serving as the fiscal sponsor to the Beginnings Fund. It provides fiduciary oversight, financial management, and other administrative functions to the Fund, including legal and administrative services, donor relations, and communications support.

Pathway to Impact

Healthy Pregnancy

Support availability and uptake of preventive interventions most likely to impact mortality, based on country-specific context:

- **Key diagnostics and management** of infections, anemia, and pre-eclampsia
- **Point-of-care ultrasound**

Intrapartum Care

Improve the quality of intrapartum care through rollout of evidence-based bundles:

- Post partum hemorrhage (PPH): **PPH uterotonics: oxytocin, heat-stable carbetocin, misoprostol, tranexamic acid, and detection drapes**
- Sepsis: **intrapartum azithromycin**
- Prematurity: **antenatal corticosteroids**
- Prolonged/Obstructed Labor**: intrapartum monitoring, safe caesarean section, and neonatal resuscitation**

Small and Sick Newborn Care

Improve quality of care for pre-term and low birth weight babies through scale up of neonatal care at district level.

- Prematurity**: Level 2 Care plus CPAP and iKMC:** phototherapy, radiant warmers, syringe pumps, pulse oximeters, O2 concentrators, hemoglobinometers, suction machines, glucometers, and caffeine citrate
- Sepsis: **in-patient management of Possible Serious Bacterial Infection (PSBI).** *Community-level interventions will be included based on government priorities*

Products: deploy reliable supply of proven innovations at lower-cost, higher volumes, and improved quality and uptime

Workforce: post-graduate training, onsite mentorship, and increased staff to reach adequate numbers and skill-mix

System Enablers: data, quality improvement, emergency referrals and transport, and advocacy for financing and scale

Beginnings Fund will invest in the scale-up of proven interventions at the health facility level to address mortality drivers during pregnancy, labor and delivery, and the newborn period. Investments will target improved supply and demand for lower-cost, higher-quality **products**; strengthen health **workforce** quality, beyond routine didactic training and expand the maternal and newborn health workforce; and support critical health **system** enablers, including data systems, quality improvement and emergency transport and referral necessary to deliver quality outcomes that countries can sustain.

The Beginnings Fund will support the following interventions based on country context and priorities, including Every Newborn Action Plan (ENAP), and Strategies for Ending Preventable Maternal Morbidity and Mortality (EPMM) acceleration plans, and considering existing or planned government support and other partners' support.

Healthy pregnancy: Quality antenatal care through increased access to **point-of-care diagnostics** to support **early detection** and **early management of complications.** Early identification of **infections, anemia and high blood pressure**, and associated treatment, will help reduce sepsis, postpartum hemorrhage, and eclampsia. **Point-of-care ultrasound** will increase demand and retention in antenatal care, date gestational age, and help identify and refer high-risk pregnancies.

Intrapartum care: Reliable, safe, and timely intrapartum care. The **postpartum hemorrhage (PPH) bundle** for early detection and management will be rolled out to address this leading cause of maternal mortality. This bundle includes the use of the calibrated drape, for timely and accurate identification of severe bleeding, and treatment with high-quality uterotonics (oxytocin, misoprostol, tranexamic acid, and integration of heat-stable carbetocin). To reduce obstructed labor which drives maternal morbidity, stillbirths, and neonatal encephalopathy, **digital intrapartum monitoring** will better detect maternal and fetal distress to enable timely emergency obstetric and newborn care. Improved uptake of **antenatal corticosteroids** during pre-term labor will reduce respiratory distress in premature babies. To prevent maternal sepsis, introduction of **maternal azithromycin** during labor will be supported. The number of **midwives**, **anesthetists, specialist clinical officers, and obstetrician/gynecologists** will

be increased through post-grad training to improve the quality of care. Onsite EmONC mentoring, including mentoring of obstetric surgical teams will improve the quality and timeliness of care in hospitals. Delays in access to care will be reduced through investments in **emergency transport and referrals**. Trained and equipped **neonatal resuscitation** teams will be present in all emergency deliveries.

Small and sick newborn care: To improve the quality of care for babies who are born too soon or sick, increased usage of Continuous Positive Airway Pressure **(CPAP)** machines to treat respiratory distress syndrome, alongside **immediate Kangaroo Mother Care (iKMC)** for all pre-term/low birth weight infants will be supported. Sites will be fully equipped with the essential technologies and medicines for Level 2 care. The number of neonatal nurses, specialist clinical and medical officers, pediatricians, neonatologists, and biomedical technicians to improve care and reduce rotation of trained staff will be increased. Support will be given to facilities to shift the organization of care in delivery rooms and neonatal units in order to reduce separation for mothers and newborns, especially for all premature and low birth weight babies, so they are warm, protected against infection, and have improved breastfeeding and respiratory stability.

Underlying all of this is the development of effective management systems, which will ensure clear protocols are established and followed, reinforced by data mechanisms that manage and monitor all pregnant women and newborns in the target facilities. Continuous review and improvement will be essential through active and timely use of data at all levels to identify areas requiring increased supervision, process improvements, and system maintenance.

Technical and Operational Plan

The Beginnings Fund will support up to 10 priority countries to achieve national goals of accelerating the rate of decline in maternal mortality, stillbirths, and neonatal mortality. The fund plans to support focus countries in two phases from 2025-30. Phase 1 countries, Kenya, Ethiopia, Malawi, Rwanda, Ghana, Tanzania, and Zimbabwe will begin in 2025, with Malawi in Q2, Rwanda and Tanzania in Q3, and Ethiopia, Ghana, Kenya, and Zimbabwe in Q4. These countries were selected based on having a high burden combined with high institutional deliveries, government readiness and ability to move quickly, and strong relationships between the founding donors and national partners. Phase 2 countries, Lesotho and Uganda will begin in 2026. Planning in Nigeria will begin in 2027, with increased support planned for Fund Two (2031-2035).

Beginnings Fund supports the reduction in maternal and neonatal mortality and stillbirths by:

1. **Investing in governments and organizations** that are implementing prioritized, evidence-based interventions in high-burden districts to increase effective coverage; increasing the supply and quality of MNH products; expanding health workforce training, and strengthening data ecosystems nationally and regionally;
2. **Complementing** existing/planned investments for greater reach;
3. I**nfluencing** scale-up through sharing demonstrated results and advocacy.

Invest: Beginnings Fund will invest in the scale up of priority, evidence-based interventions to strengthen implementation of country acceleration plans to reduce maternal and newborn mortality and prevent stillbirths. Investments will focus on high-burden districts where ~50% of maternal deaths, stillbirths, and neonatal deaths occur, estimated at 10 district hospital referral networks per country, including central hospitals in large urban districts. Investments will support interventions across products, workforce, and systems to be layered in target facilities to strengthen antenatal, intrapartum, and small and sick newborn care. Through a co-design process, Beginnings Fund will engage with national and district leaders to develop a country investment plan. The investment team will build from the founding donors' strong government and stakeholder relationships and implementing partner networks to conduct

in-country scoping led by the MOH to develop investments and identify leaders with the political will to drive change at the national or district level. Implementing partners will be organized as a consortium, working towards one results framework under the oversight of the MOH. Beginnings Fund will also consider direct investments to government agencies as appropriate. Beginnings Fund investments at the facility level will increase coverage of key interventions to >80% and strengthen the MNH ecosystem for products, workforce, and systems at national and regional level. Investments in national and regional organizations will drive down cost and improve availability of essential products, improve and expand the MNH workforce, support national data systems to enable monitoring of MNH interventions, and expand national emergency transport and referrals systems. Together these investments will lead to significant institutional maternal and neonatal mortality and stillbirth reductions over 2025-2029 (see Annex A), and over 300,000 lives saved across nine countries (excludes Nigeria).

Complement: Beginnings Fund will seek greater impact by aligning efforts with government plans and multi- and bi-lateral funding partners, building on existing maternal and newborn strategies supported by the World Bank and Global Financing Facility, USAID's Momentum initiatives, FCDO's midwifery workforce investments, and WHO, UNICEF and UNFPA support for ENAP/EPMM acceleration plans. Five of the nine targeted countries are supported by NEST360. Additional Beginnings Fund investment will add the maternal component to existing or planned district-level neonatal support and gain synergies around data systems, clinical and biomedical mentoring, and quality improvement.

Influence: Beginnings Fund will negotiate government commitments such as co-financing or in-kind contributions upfront, engaging the Presidency and/or Ministry of Finance, in addition to the Ministry of Health, to ensure the political commitment necessary for adoption of interventions, absorption of workforce and operating costs, and further scale. A key measure of success for the fund will be its ability to catalyze further investment from the public, private, and philanthropic sectors. Demonstrating early results in Phase 1 countries is critical to generate additional interest and investment for further national and regional scale. All Phase 1 countries have >80% of women giving birth in health facilities, and most deaths happening in facilities, allowing for national reduction of mortality rates if significant coverage of key interventions is achieved. Beginnings Fund will advocate with national and sub-national leaders for the additional infrastructure, equipment, supply chain, and workforce investments needed, as well as increasing girls' education completion and contraception coverage, access to legal and safe abortion, and strong primary health care systems for improved maternal and newborn outcomes in partnership with WHO, UNICEF, and UNFPA country offices. The fund will actively seek to crowd in other private donors for joint investments. These efforts will not only sustain Beginnings Fund investments but also scale interventions to deliver impact at the national level.

Objectives and Activities

Overarching Objective: In partnership with governments, invest in the scale-up of evidence-based interventions to address the primary drivers of maternal mortality, stillbirths, and neonatal mortality in high-burden facilities, to:

1. Deploy proven MNH **products** including diagnostics, medicines, devices, and supplies at lower cost, higher – volume, and proven quality;
2. Strengthen and expand the quality, number, and distribution of the (MNH) **workforce;**
3. Strengthen other critical health **system** enablers, including data and quality, emergency transport and referral systems, and advocacy for greater impact.

A snapshot of key activities the Beginnings Fund will invest in is provided below, with a more detailed list in Annex C and a pipeline of potential grantee-partners in Annex G.

Facilities in high-burden districts:

- Target 90 high-burden referral networks of national hospitals (25), district hospitals (90), sub-district hospitals (246), and health centers (490).
- Equip facilities with essential products and devices, and reduce stockouts and wastage.
- Mentor clinical and biomedical staff on-site, to increase uptake of antenatal, intrapartum, and SSNC interventions, and institutionalize EmONC/SSNC skill drills. Employ additional MNH staff, via government, but with salary support on a two- to three-year basis until absorbed.
- Install MNH inpatient data systems and use data to improve practice. Provide catalytic infrastructure and equipment to improve care.

National level:

- Work with governments and centralized medical stores/pharmaceutical supply agencies to ensure sustainable access to high-quality products by strengthening capacity in forecasting, procurement, sustainable financing, quality supply security, and demand. This may include standing up a unit to rapidly address current market barriers to the scale-up of priority antenatal, intrapartum and SSNC products and set-up strategy, systems and processes to do this continually as new products emerge.
- Consolidate demand across government buyers (pharmaceutical supply agencies, MOH) to gain more value from current spend. Incentivize high-quality product suppliers to enter markets through innovative financing mechanisms. Support regulatory reform to reduce fragmentation and support quality. Build a logistics coalition to limit waste, reduce costs, and improve distribution.
- Update essential medicines lists and clinical guidelines to increase uptake of priority interventions and roll out training. Support districts to budget for products costs and equipment maintenance.
- Plan for priority MNH cadres with MOH, training institutions, regulatory councils and professional associations. Improve deployment and retention by addressing accreditation, scope of practice, licensure, career paths, and rotation practices.
- Strengthen and expand MNH training programs for midwives, obstetrician/gynecologists, anesthetists, neonatal nurses, pediatricians, and neonatologists through university and hospital-based programs.
- Support scholarships, faculty and curricula development, and competency-based approaches. Improve teaching hospitals' maternity and newborn units by providing equipment and supplies, clinical educators/preceptors, and simulation/skills labs, and support quality improvement practices.
- Support national rollouts of emergency transport and referral systems, using proven innovations such as m-mama, alongside strengthened referral protocols

Regional level:

- Support distribution and maintenance of rugged MNH devices which meet UNICEF Target Product Profiles.
- Conduct regional market shaping activities to aggregate demand for MNH product bundles across countries in order to negotiate favorable prices and better supply and quality.
- Roll out hospital-based training for specialist cadres such as midwives and pediatricians in the region.
- Increase capacity for neonatal nurse training in the region by building faculty, curricula, and certification processes.
- Support frontline neonatal providers in the region to take-up effective practices such as iKMC.

Data, Evidence Generation, and Learning

In partnership with governments and national experts, the Beginnings Fund will support the generation of core maternal and newborn inpatient data indicators in ~361 targeted hospitals (25 national/regional hospitals, 90 district hospitals, and 246 sub-district hospitals) providing antenatal, EmONC and small and sick newborn care. In four Phase 1

countries, the core dataset will be tested and refined following data collection and implementation of quality-of-care indicators for intrapartum and neonatal care prior to national adoption. Coverage of Beginnings Fund priority interventions will be measured as outlined in Annex A. Analysis of intervention coverage and institutional maternal and neonatal mortality and stillbirth rates in targeted districts and at the national level will be reviewed quarterly and annually to assess impact, extrapolating from facility-level indicators to model national level impacts on mortality.

Beginnings Fund will convene relevant stakeholders, national experts from Phase 1 and 2 countries, and global experts to develop a minimum MNH inpatient data set, building from the existing neonatal data set developed through NEST implementation. The Director of Statistics and Impact Evaluation will lead efforts to identify a regional organization that will support countries' implementation and integration of the agreed data indicators for a core MNH data set in government commissioned electronic medical records system (EMR) or DHIS2 software and platform where EMRs do not exist. Beginnings Fund will support each country as it establishes the system in targeted districts, and will support software developers, data manager, statistician, and data clerks, and test and refine the rollout with the country preferred data partner prior to national adoption. Interactive dashboards will be co-created, tested, and refined with country MNH stakeholders to track quality of care indicators and support quality improvement initiatives. Similarly, to track workforce, commodities, and referral systems investments, Beginnings Fund will leverage existing data platforms such as human resource information systems and logistics information systems to create a unified and centralized dashboard. These user-friendly MNH dashboards enable data visualization and usage in real-time by program managers at facility, district, and national levels, to enable quality improvement and program planning and monitoring. The dashboards will be designed, developed, and integrated into DHIS2 software to include all routine and program data, including coverage of key interventions, EmONC functionality, staff:patient ratios, retention of trained staff, staff competency, patient satisfaction, stockout rates, availability of products and supplies, and equipment uptime. All data will be generated as part of routine data to support the provision of quality care in hospitals, and health system functioning and planning as part of Government mandates.

As part of each country plan, we will have a country dashboard that pools data of all relevant indicators as outlined in the MEL list of indicators. In addition, we will have an online multi-country interactive dashboard that will be developed by the multi-country evaluation partner reporting all indicators in the pathway to impact. Data will be drawn from country level dashboards, updated frequently (aiming for real-time), and accessible to all partners. Updates of the indicators will be shared every quarter and detailed analytical outputs on implementation progress, coverage, and intermediate outcomes will be provided annually. Beginnings Fund will generate critical evidence through iterative refinement of the transition tool in Phase 1 countries and WHO will develop guidance on optimal organization of maternal and newborn care for country impact, as a global good to be implemented in Phase 2 countries and beyond. The fund will support inter-country learning to enable the rapid adoption of effective practices.

Overall Budget and Sources of Funds

The Beginnings Fund's Fund One budget is USD525million for a period of 76 months ending 31 December 2030. Of this budget, USD495million is for regional and country investments and measurement, evaluation, and learning, and USD29.9million is for operations and fiscal sponsorship fees at 5% of the total fund (see Annex D for summary budget). Of the USD495million available for investment, USD46.5million is allocated to regional investments to support multi-country/continent-wide activities; USD435million for country implementation (5% of which will support in-country measurement, evaluation, and learning); and USD13.5million for cross-country measurement, evaluation, and learning. Funds are allocated to each country based on population, time to impact, government leadership, opportunity to leverage additional resources, political stability, and ability to absorb workforce investments.

Should Fund One demonstrate initial success, the founding donors are willing to anchor Fund Two at similar amounts.

Risk Analysis

Key Risks (Impact/Likelihood)	Mitigating Strategies
Organizational Beginnings Fund does not raise the targeted USD525million. (Medium /Medium)	Country implementation will be phased in to ensure only available funds are programmed. The committed USD476.1million have been raised to date and will support regional investments, Phase 1 implementation, and measurement, evaluation, and learning. Phase 2 implementation will begin in 2026 in Lesotho and Uganda if the remaining USD47.9million is raised. The fund will adopt a lean staffing structure and leverage expertise from founding donors to keep operating costs low.
Implementation Governments are not adequately prepared to absorb additional staff. Health workforce attrition and strikes would reduce quality of care. (High/Medium)	Beginnings Fund will plan with governments and negotiate upfront commitments to absorb additional staff, with milestone-based payments based on performance and planning metrics, and will support workforce plans to improve retention strategies, including addressing any risks from policies that encourage health workers towards foreign employment.
Political Change in political leadership may cause instability or shift government priorities away from the fund's objectives. (Medium /Low)	Beginnings Fund will establish strong relationships across government departments, with sub-national government and facility-level management, and with professional associations to ensure continued buy-in and implementation through changes in governance at any level of government.
Macroeconomic Shrinking fiscal space in target countries due to economic instability may reduce government's ability to invest in health systems that critically complement Beginnings Fund interventions, such as infrastructure or workforce. (High / Medium)	The fund will allocate a contingency budget in each country and allow overall country budgets a degree of flexibility to respond to changes in the funding landscape. It will leverage existing institutional funding for some bigger systems costs and negotiate a diverse set of non-financial government commitments upfront, including in-kind contributions, agreements on workforce absorption, and policy, to offset any macroeconomic risks.
Climate Change and Outbreaks Risk of disease outbreaks and natural disasters, both exacerbated by climate change. (High / Medium)	Beginnings Fund investments will be designed to remain nimble and responsive to emergency crises, including allowing program resources and activities the flexibility to pivot or adapt accordingly, for example by ensuring emergency access to maternal and newborn services continue during the event of a natural disaster.

Risk/Reward Analysis: Despite the risks, the fund has a unique opportunity to significantly change the trajectory of maternal and newborn survival, beyond what funders working in silos can achieve. The potential to save over 300,000 lives by 2030 outweighs the associated risks. At a minimum, this investment will produce 2,950 maternal and newborn specialists, improve staff:patient ratios in 115 national and district hospitals, support 20,000 existing health workers to reach EmONC and SSNC competency, and improve quality of care for an estimated 35 million lives.

Sustainability

Beginnings Fund investments in the maternal and newborn health ecosystem at the national and regional level will sustain improvements beyond the life of direct investments. Efforts to strengthen the policy, financing, and regulatory environment for workforce and products will help retain trained staff and build the pipeline and approach for ongoing access to quality and cost-effective products. Improving teaching hospital practices and competency-based teaching and learning will not only impact the quality of training in Beginnings Fund-supported investments but for future generations of health workers. The fund's willingness to support minor renovations and equipment in hospitals will provide the foundation for successful implementation. Supporting the set-up costs for a national emergency transport and referral network with philanthropic and multilateral collateral will allow government to absorb the modest operating costs to sustain the network and plan ongoing improvement. These efforts will not only serve to sustain Beginnings Fund investments but scale interventions that deliver national impact.

That said, realistically, more financing will be required beyond 2030 for additional support to realize national coverage, maintain quality, and provide more specialized obstetric and neonatal care. Thus, should Fund One show impact and pathways to sustainability, Beginnings Fund will raise a Fund Two to invest further in the 10 initial countries, and consider adding one or two more, pending budget.

Annex A: Beginnings Fund Indicators

The Beginnings Fund Investment Team developed an impact model to inform these indicators and targets.

The LiST[10] model was used to generate lives saved based on increasing coverage of priority interventions. Beginnings Fund will target health facilities where 50% of deaths occur. Based on an analysis of three Phase 1 countries, these same facilities will reach an average 30% of women and newborns accessing care across all nine countries. The total number of lives saved is 322,847, comprising 189,581 newborn lives, 21,002 maternal lives, and 112,264 stillbirths. In general, stillbirths are vastly undercounted, and more stillbirths will be accounted for at baseline with additional impact once data is improved. The model doesn't currently account for AI-enabled ultrasound, intrapartum azithromycin, and community-level management of PSBI (which may be included in individual country plans based on government priorities). If accounted for, these interventions will likely increase the impact on lives saved.

These model estimates also account for national mortality reductions from interventions to improve workforce and referrals. The mortality reduction rates will serve as conservative estimates for the institutional mortality reductions that will occur in targeted facilities. Baseline indicators and targets will be refined per country, as the Investment Team develops country plans and collects more data on the ground.

Beginnings Fund - Impact Indicators

	Kenya	Tanzania	Rwanda	Malawi	Lesotho	Ethiopia	Zimbabwe	Ghana	Uganda	Total
In targeted facilities where 50% of deaths occur										
Lives Saved										
Maternal lives	4,900	3,856	895	1,795	263	4,107	1,211	1,141	2,834	21,002
Newborns<1 months	25,432	33,633	9,027	11,864	2,857	54,170	12,913	14,075	25,610	189,581
Stillbirths prevented	16,162	26,008	4,663	7,102	1,216	27,847	5,958	8,694	14,614	112,264
Total Lives Saved	**46,494**	**63,497**	**14,585**	**20,761**	**4,336**	**86,124**	**20,082**	**23,910**	**43,058**	**322,847**
Potential impact on national mortality rates, if interventions are scaled nationally*										
2024	530	238	259	381	566	267	357	263	284	
2029	432	183	205	297	476	196	288	218	225	
% reduction in MMR	**18%**	**23%**	**21%**	**22%**	**16%**	**27%**	**19%**	**17%**	**21%**	
2024	18	20	18	19	35	26	25	23	19	
2029	12	12	11	12	23	15	16	14	11	
% reduction in NMR	**36%**	**39%**	**39%**	**36%**	**36%**	**44%**	**38%**	**37%**	**40%**	
2024	19	18	17	16	27	21	19	21	15	
2029	16	15	15	13	23	16	17	19	12	
% reduction in SBR	**14%**	**16%**	**11%**	**16%**	**14%**	**21%**	**15%**	**12%**	**20%**	

*Reductions in national mortality are conservative proxy indicators for institutional mortality reductions in targeted facilities

[10] The Lives Saved Tool (LiST) estimates the impact of scaling up maternal, newborn, and child health, and nutrition interventions in low- and middle-income countries. LiST calculates changes in cause-specific mortality based on intervention coverage change, intervention effectiveness for that cause, and the percentage of cause-specific mortality sensitive to that intervention. Coverage data come from Demographic and Health Surveys and Multiple Indicator Cluster Surveys, as well as WHO and UNICEF. Default effectiveness values come from systematic reviews, meta-analyses, Delphi estimations, and randomized control trials. Baseline mortality is from country-level estimates from WHO, UNICEF, UNFPA, World Bank Group, and the UN Inter-agency Group for Child Mortality Estimation.

Annex A: Key indicators and targets	Short term outcomes (0-2 years)	Medium-term outcomes (3-5 years)
Coverage of Evidence-Based Interventions	**Facility Level:**	
	Target: 50%	Target: 70% by Year 3; >80% by Year 5
	• % of pregnant women attending antenatal care (ANC) screened for anemia, infection, high-blood pressure • % treated for anemia, infection and high-blood pressure • % of women attending ANC who receive point-of-care ultrasound, with gestational age recorded • % of women who receive digital intrapartum monitoring in labor • % of eligible women with pre-term labor who receive antenatal corticosteroids • % of women eligible for parenteral antibiotics who receive them • % of women with PPH who receive early detection of blood loss (300ml-500ml) and receive WHO-recommended treatment bundle • % increase in c-section rate at CEmONC sites to at least 15% of deliveries • % reduction in the rate of surgical site infection post c-sections by >16% • % of eligible babies who receive neonatal resuscitation • % of pre-term and low birth weight babies who receive KMC • % of babies between 1,000g and 1,499g receiving prophylactic CPAP • % of babies between 1,500g and 1,999g with respiratory distress symptoms receiving CPAP • % of babies with clinical jaundice receiving phototherapy • % of babies with suspected sepsis who receive parenteral antibiotics • % of babies not hypothermic on admission to the neonatal unit • % of babies not hypothermic at any time in the neonatal unit	
Products	**Facility-level:**	
	Target: 100% in Phase 1 countries	Target: 100% in Phase 2 countries
	• % of targeted facilities equipped with appropriate bundle of MNH products Hospitals: ○ ANC package: Point of care diagnostics, BP, hemoglobin, urinalysis, ultrasound ○ Intrapartum package: Digital monitoring device (CTGs), ultrasound, PPH bundle: carbetocin/oxytocin, misoprostol, tranexamic acid, detection drapes, appropriate beds, azithromycin, radiant warmer, emergency carts, consumables ○ Small and Sick Newborn package: CPAP, BiliDx, phototherapy, radiant warmers, syringe pumps, pulse oximeters, oxygen concentrators, hemoglobinometer, suction machines, glucometers, caffeine citrate, consumables Health centers: ○ ANC package ○ Intrapartum package	
	Target:>70% availability	Target: >90% availability
	• Increase in newer MNH commodities (detection drapes, heat-stable carbetocin)	
	Target: 5-10% in Year 1; 10-20% in Year 2	Target: 30-50% (less than 5-10% stockouts)
	• % reduction in stockouts of essential MNH products in each period.	

	Target: 5-10% in Year 1; 10-20% in Year 2	Target: 50%
	• % increase of MNH products orders that meet or exceed the lead-time given when order was placed	
	National level:	
	Target: 100% of Phase 1 countries	Target: 100% of Phase 1 and 2 countries
	• Clinical guidelines updated and regulatory approval received for key MNH products (point-of-care ultrasound, maternal azithromycin, heat-stable carbetocin, detection drapes)	
	Target: 5-10% in Year 1; 10-20% in Year 2	Target: 30%
	• % increase of essential MNH products available in national medical store	
	Target: 50% – 90% on time in full deliveries	Target: 90-95% on time in full deliveries
	• % of MNH products orders fully satisfied as per the demand without backorders or partial fulfillment.	
	Target: Improve by 10% - 30%	Target: Improve by 50%
	• variance between the forecasted and actual demand for MNH products	
	Target: 10-20% in Year 1; 20-30% in Year 2	Target: 30-50%
	• % reduction of Substandard and Falsified MNH products on the market	
	Target: Countries establish procurement criteria by Year 3 ○ Tier 1: SRA approved, WHO prequalified, or assessed low risk by an Expert Review Panel ○ Tier 2: Locally registered and have GMP certification ○ Tier 3: All others that are NDRA registered	
	• Updated procurement selection criteria on quality and price / purchasing decisions, to increase access to higher quality products	
	N/A	Target: TBD at country level
	• Increased budget allocated and disbursed for quality MNH products	
	Regional level:	
	Target: 10-20% in Year 1; 20-30% in Year 2	Target: 30-50%
	• % increase in volumes of quality MNH products ordered before and after interventions	
	Target: rationalized by end 2026	N/A
	• Alignment on PPH bundle SKUs and other MNH products across guidelines, quantifications, and procurements	
	Target: 100% of Phase 1 countries	Target: 100% of Phase 1 and 2 countries
	• Increased number of countries procuring high-impact MNH products, and procurement guidelines balance quality and price	
	Target: 100% of Phase 1 countries	Target: 100% of Phase 1 and 2 countries
	• Sustainable financing plan for focus products developed and implemented	
Workforce	**Facility level:**	
	Target: 5,000; 60% of staff	Target: 20,000; 80% of staff
	• # and % of health workers in maternity and newborn units reaching EmONC and SSNC competency	
	N/A	Target: 80%

	• % of targeted hospitals have requisite skills-mix of MNH specialists (midwives, neonatal nurses, anesthetists, pediatricians, neonatologists, biomedical technicians) defined by country • % of targeted hospitals achieving targeted staff:patient ratio in targeted newborn units • % of targeted hospitals achieving targeted staff:patient ratio in targeted maternity units • % of targeted health centers achieving targeted ratios for midwives on staff	
	Target: 500 employed	Target: 2,500 employed; 90%
	• # of unemployed health workers employed in targeted facilities and % absorbed by government on agreed timelines	
	National level:	
	N/A	Target: 2,950; 90% deployed appropriately; 95% retained

	• # of health workers completing post-basic or postgraduate training to become maternal and newborn specialists (midwives, neonatal nurses, specialized clinical/medical officers, anesthetists, pediatricians, OB/GYNs, neonatologists)
	  ○ % deployed appropriately to targeted maternity/neonatal units
	  ○ % retained 12 months post-completion	

	N/A	80%
	• % increase in number and proportion of maternal and newborn health specialists in the public sector by cadre	
	N/A	95%
	• % of targeted MNH training programs integrating simulation into teaching, schedule, and assessment • % of targeted MNH training institutions providing team-based training of skills-mix of trainees, and lecturers across MNH cadres and departments	
	Regional level:	
	Target: 2 additional	Target: 5 additional (cumulative)
	• # of neonatal nurse training programs in targeted region intaking trainees	
	Target: 200	Target: 1,000 (cumulative)
	• # of nurses providing care in neonatal units complete online training and receive national certification	
	Target: 10; Kenya, Tanzania, Rwanda	Target: 20; + Uganda, Ethiopia, Zimbabwe, Malawi
	• # of hospitals and countries (outside universities) training pediatricians in targeted countries	
Systems	**Facility and District-level**	
	50% of targeted districts in Phase 1 countries	90% of targeted districts in Phase 1 and 2 countries
	• % of facilities that meet the essential level of emergency referral readiness • % of facilities that practice quality improvement • % of facilities with 24/7 family access to newborn inpatient unit for duration of stay	
	50% of facilities and districts in Phase 1 countries	100% of facilities and districts in Phase 1 and 2 countries
	• % of facility and targeted districts that utilize MNH dashboard to make decisions and practice quality improvement • % of targeted districts budgeting for essential MNH products costs based on actual need • % of targeted districts with at least one Level 2 inpatient unit plus CPAP	
	National-level	
	50% in Kenya	90% in Kenya, Malawi, Rwanda

	• % of pregnant women and newborns able to access a well-functioning EmONC facility within 2 hours travel time • % of Basic EmONC facilities within 30 minutes of a Comprehensive EmONC facility	
	50% of Phase 1 countries	90% of Phase 1 and 2 countries
	• % of countries significantly increasing MNH investments and budgeting for recurring costs • % of countries with increasing rollout of priority interventions in additional districts, with domestic resources/co-financing • % of countries allocating domestic resources for priority MNH product procurement	

Annex B: Beginnings Fund Governance and Leadership Teams

Investment Committee

Faustina Fynn-Nyame, BSN
Executive Director, Africa, Children's Investment Fund Foundation (CIFF)
Over 16 years of experience in sexual reproductive health and private sector health care delivery in Europe and Africa. Former Director at Women's Health Project and former Country Director at Marie Stopes International.

Robyn Calder, MSc, MBA
President, The ELMA Philanthropies Services (U.S.) Inc.
Board Member, The END Fund
13 years of experience leading The ELMA Philanthropies and early work experience with Clinton Health Access Initiative and CIFF.

Dr. Paulin Basinga, MD, MS, PhD
Director, Africa, Gates Foundation
20 years of experience strengthening health systems, research, delivery, and partnerships in HIV, TB, maternal and child health, and advocacy in Africa and globally. Former Senior Advisor at the Rwanda Biomedical Centre, in Rwanda's Ministry of Health.

Elizabeth Tanya Masiyiwa, MSc, MBA
President and Chief Executive Officer, Delta Philanthropies and Higherlife Foundation
Non-Executive Director, Econet Wireless Zimbabwe
Seven years of experience at Delta Philanthropies; co-founder and Director of STAR Leadership Academy; and CEO of Akello, an e-learning platform

Tala Al Ramahi, MA
Director, Presidential Court UAE
12 years of experience leading public policy initiatives for the UAE government, including serving as Chief Strategy Officer for Reaching the Last Mile and the Special Olympics World Games Abu Dhabi 2019.

Investment Team

Alice Kang'ethe, MSc, Chief Executive Officer
Former Chief Operating Officer, Clinton Health Access Initiative
Over 18 years' experience in global health programs strategy and execution in Africa, Sout East Asia, and Latin America

Anne Magege, MSc, Program Director
14 years of experience building the health investment portfolio at ELMA. Seven years in health program implementation on the continent.

Patricia Wamala, MA, Program Director
Senior Program Management specialist with over 20 years of experience in international development, with a specific focus on reproductive, maternal, newborn and child health.

Professor Eric Ohuma, MSc, PhD, Director of Statistics and Impact Evaluation
Professor of Medical Statistics and Epidemiology at the London School of Hygiene & Tropical Medicine, UK.
15 years of experience in maternal, newborn, and child health and impact evaluation of complex interventions in Africa.

Dr. David Gathara, MSc, PHD, Program Measurement and Evaluation Manager
Assistant Professor, Health Systems, London School of Hygiene and Tropical Medicine
Nursing graduate and epidemiologist with 15 years of experience in health systems research and evaluation, with a focus on health workforce and newborn care.

Jingyi Song, MIA, Senior Program Officer
12 years of experience developing and managing health investments at ELMA Philanthropies and health programs in Africa at Clinton Health Access Initiative.

Fred Lwegaba, FCCA, Director Finance and Operations
Over 30 years' experience in finance and business strategy across nonprofit, private, and public sectors. Executive and corporate services expertise and leadership in healthcare, energy, and taxation industries.

Flavia Kibor, BSc, Executive Assistant
10 years' experience in administration and program management, most recently with WHO in the Sexual and Reproductive Health and Emergency Preparedness and Response Clusters

Jess Flamholz, BA, Head of Investor Relations
14 years' experience in local and global nonprofit fundraising, donor and stakeholder relations, and community engagement, also serving as a philanthropy advisor at Geneva Global.

Technical Advisory Group

Dr Sanjana Bhardwaj, MD, MPH, FFPH
Deputy Director, Women's Health and Primary Health Care, Policy Advocacy and Communications, Gates Foundation
30 years of public health experience globally and in Asia Pacific, North America, and Africa

Rhobhi Matinyi, MPH
Director & Portfolio Lead, Health & Climate, Children's Investment Fund Foundation (CIFF)
15 years of healthcare experience in Africa and Asia as Co-founder & Chief Strategy Officer at Epione, and Associate Partner at Dalberg

Meron Makonnen
General Counsel, ELMA Philanthropies Services
15 years of experience providing legal and strategic advice on program and institutional development, as well as political and economic risk

Tej Nuthulaganti, MS, MPH
Head of Global Health & Impact, Mohamed Bin Zayed Foundation for Humanity
Team Lead & Senior Specialist, Health Workforce at The Global Fund
16 years of health policy and program implementation experience in Africa

Silvia von Löwis of Menar
Deputy Director, Product Introduction, Maternal, Newborn, Child Nutrition and Health, Gates Foundation
17 years of product introduction experience with large-scale pharmaceutical companies, previously heading up the Africa Public Sector Team in Novartis.

Rebecca Mwabvu, MEc, MPM, ACP
Head of Global Health, Higherlife Foundation; Maternal and Newborn Health Lead, Delta Philanthropies
Over 10 years' experience in health program design, grant management and social development

Annex C: Summarized Operational Plan

	Facilities in High-Burden Districts	National Level	Regional Level
Products	Equip facilities with technologies, medications, diagnostics and supplies for antenatal care (ANC), intrapartum, and small and sick newborn care (SSNC). • 490 health centers: ANC and intrapartum package. • 246 sub-district hospitals: ANC, intrapartum, and SSNC. • 90 district hospitals: ANC, intrapartum, and SSNC. • 25 regional/national hospitals: ANC, intrapartum, and SSNC. Rollout evidence-based process improvements such as stocked emergency trolleys. Improve forecasting and monitoring of MNH products to reduce stockouts and wastage. Track equipment uptime and preventive maintenance.	Increase negotiating power for government buyers by consolidating funding across match mechanisms and existing spend. Aggregate demand and improve procurement to achieve pooling and program alignment, narrowing product selection (established) and elevating higher efficacy products (emerging). Vet, select, and negotiate orders to obtain a high-quality and affordable supply of prioritized MNH products. Incentivize high-quality suppliers to enter national markets and attain volume by providing liquidity to improve financial terms, enabling price reduction through buy-downs or innovate service or subscription business models, driving out lower quality products. Support pharmaceutical supply agencies to maintain shifts through regulatory reform and information availability to sustainably protect the market, reduce wastage, and obtain better quality products. Build a logistics coalition to reduce the # of intermediaries to limit waste, reduce costs, and improve distribution. Support demand creation and uptake by updating essential medicines and devices lists, clinical guidelines, and supporting national training rollout. Support MOH (national and sub-national) budgeting for ongoing supply, consumables, and maintenance. Within national health information system, and through rapid surveys, conduct ongoing monitoring to evaluate impact and improve overall value chain performance.	Support sales, distribution, and maintenance of rugged MNH devices which meet UNICEF Target Product Profiles. Conduct regional market shaping activities to aggregate demand for MNH product bundles across countries to negotiate favorable prices and better supply and quality.

	Facilities in High-Burden Districts	National Level	Regional Level
Workforce	Train and mentor clinical and biomedical staff on-site to increase uptake of ANC, intrapartum, and newborn bundles. Support staff rotations at best practice sites. Train neonatal resuscitation teams for all deliveries. Mentor obstetric surgical teams to improve quality and reduce infections. Institutionalize EmONC/SSNC skill-based practice and utilize digital tools to improve practice. Fund districts for additional MNH staff with milestone-based payments.	Plan priority MNH cadres and address bottlenecks to optimally deploy and retain staff (accreditation, scope of practice, licensure, career paths, rotation practices). Strengthen and expand pre-service and post-graduate MNH workforce training programs at university and hospital level for: midwives, OB/GYNs, specialist clinical and medical officers, anesthetists, neonatal nurses, pediatricians, neonatologists. Partner with government and nonprofit employers to release trainees on salary and bond. Target selection and deployment to priority districts/facilities. Provide scholarship support, faculty, and curricula development, improve simulation and competency-based approaches and team training. Improve teaching hospitals' maternity and newborn units: equipment and supplies, clinical educators/preceptors, simulation/skills labs, and quality improvement. Consider time-limited post-graduation salary support to facilitate rapid employment. Support post completion mentoring. Facilitate uptake of additional maternal and newborn staff through technical support to MOH to target and utilize Global Fund support. Track changes in workforce absolute numbers, distribution, competencies, and ratios at both facility and national level.	Roll out hospital-based training for specialist cadres such as midwives and pediatricians in the region. Increase capacity for neonatal nurse training in the region by building faculty, curricula, and certification processes. Support frontline neonatal providers in the region to take-up effective practices such as iKMC.

	Facilities in High-Burden Districts	National Level	Regional Level
Systems	Conduct facility assessments to identify gaps in HR, products, practices, equipment, and infrastructure. Set up maternal and newborn inpatient data systems in targeted facilities and establish baselines. Provide catalytic infrastructure and equipment to improve care for mother-baby dyad, warm chain, and mother-baby neonatal units for expanded KMC. Support quality improvement and data use to shift practices to improve care. Improve emergency stabilization, communication, referral, and transport protocols.	Work with DHIS2 managers to build key maternal and newborn health and product and workforce indicators into national health information system at outset. Support utilization of MNH data for quality improvement at facility, sub-national, and national levels, and align with perinatal mortality review processes to include near misses and stillbirths. Develop national change package of tested implementation practices for scale. Strengthen national rollout of emergency transport and referral systems, using proven innovations such as m-mama, to transfer mothers and newborns in a safe and timely manner Improve national and sub-national referral management practices, including improved stabilization and monitoring of patients, and referral of high-risk women to CEmONC facilities pre-delivery. Co-fund infrastructure improvements with governments including the renovation and equipping of maternal and neonatal units to WHO norms, provision of critical supportive infrastructure such as backup power and WASH and oxygen, to improve safe delivery, neonatal care, and facilitate iKMC. Support governments at national and sub-national levels to prioritize, adequately plan, and budget for maternal and newborn interventions to ensure long-term sustainability	Develop core MNH data set to generate key quality of care coverage indicators in partnership with HISP groups and country experts. Set-up and operate MNH data system within national DHIS2 to analyze inpatient data, including dashboard visualization at program, multi-country, country, and facility level. Support utilization and improvement of the transition tool in Phase 1 countries to establish WHO guidance on optimal organization of MNH care. Spread implementation change packages through a robust learning system. Build a strong advocacy network to work with policy- and decision-makers, using data and learnings from the investments to increase attention, build momentum, and gain political will necessary for scale. Engage key civil society actors, including parent advocates, the White Ribbon Alliance, and clinical leaders.

Annex D: Summary Budget and Sources of Funds

(as at July 2025)

Budget		USD							
		4 months ending Dec 2024	2025	2026	2027	2028	2029	2030	Total
INVESTMENTS									
Regional Investments		**-**	**8,203,363**	**12,065,068**	**11,433,197**	**11,214,845**	**2,994,026**	**637,500**	**46,548,000**
Country Investments	a.								
Malawi		-	11,560,703	8,382,439	8,431,901	5,838,827	4,378,584	1,407,545	40,000,000
Rwanda		-	5,300,000	2,187,500	8,750,000	8,312,500	6,550,000	3,900,000	35,000,000
Tanzania		-	8,500,000	7,437,500	27,625,000	20,187,500	14,875,000	6,375,000	85,000,000
Ethiopia		-	3,000,000	24,541,667	26,125,000	23,458,333	11,958,333	916,667	90,000,000
Kenya		-	-	18,791,667	23,250,000	21,208,333	15,500,000	1,250,000	80,000,000
Zimbabwe		-	1,500,000	1,312,500	4,875,000	3,562,500	2,625,000	1,125,000	15,000,000
Ghana		-	-	9,333,333	12,000,000	10,000,000	6,833,333	1,833,333	40,000,000
Lesotho		-	-	1,500,000	1,583,333	1,083,333	666,667	166,667	5,000,000
Uganda		-	-	3,062,500	11,812,500	10,062,500	7,437,500	2,625,000	35,000,000
Nigeria	b.	-	-	-	3,333,333	3,666,667	2,333,333	666,667	10,000,000
Sub-total Country Investments		**-**	**29,860,703**	**76,549,106**	**127,786,068**	**107,380,494**	**73,157,750**	**20,265,879**	**435,000,000**
Measurement, Evaluation and Learning	c.	**-**	**2,250,000**	**2,060,417**	**4,547,917**	**2,372,917**	**1,556,250**	**712,500**	**13,500,000**
Total Investments		**-**	**40,314,066**	**90,674,591**	**143,767,182**	**120,968,256**	**77,708,027**	**21,615,879**	**495,048,000**
Total Beginnings Fund Operations	g.	**393,596**	**4,317,533**	**5,276,751**	**4,845,971**	**5,027,379**	**5,353,434**	**4,737,337**	**29,952,000**
Total Budget		**393,596**	**44,631,598**	**95,951,342**	**148,613,153**	**125,995,635**	**83,061,460**	**26,353,216**	**525,000,000**

Sources of Funds		USD							
		4 months ending Dec 2024	2025	2026	2027	2028	2029	2030	Total
ELMA Foundation		-	15,000,000	25,000,000	20,000,000	20,000,000	20,000,000	-	100,000,000
Gates Foundation		10,000,000	20,000,000	20,000,000	20,000,000	20,000,000	10,000,000	-	100,000,000
Children's Investment Fund Foundation		13,000,000	20,000,000	20,000,000	20,000,000	20,000,000	7,000,000	-	100,000,000
Mohamed bin Zayed Foundation for Humanity		-	25,000,000	30,000,000	25,000,000	25,000,000	20,000,000	-	125,000,000
Delta Philanthropies	a.	-	1,500,000	3,900,000	2,700,000	2,700,000	2,700,000	-	13,500,000
Anonymous donors	b.	2,085,222	17,000,000	4,000,000	4,000,000	4,000,000	3,914,778	-	35,000,000
Private donors	c.	-	1,328,000	1,175,000	175,000	-	-	-	2,678,000
Interest Income	d.	110,587	748,557	-	-	-	-	-	859,144
Surplus brought forward		-	24,802,212	80,747,171	88,870,829	32,132,676	(2,162,958)	(21,609,641)	-
To be raised (surplus)		(24,802,212)	(80,747,171)	(88,870,829)	(32,132,676)	2,162,958	21,609,641	47,962,856	47,962,856
Total Sources of Funds		**393,596**	**44,631,598**	**95,951,342**	**148,613,153**	**125,995,635**	**83,061,460**	**26,353,216**	**525,000,000**

Notes

a. Delta will invest USD13.5 million directly into programming in Lesotho and Zimbabwe and not via Global Impact.

b. The total donation of USD35 million combines three separate grants from anonymous donors (USD10million, USD10million, and USD15 million). The USD15 million donation was received in full in February 2025.

c. Includes:
USD1 million from Horace W Goldsmith Foundation
USD525,000 from The Patchwork Collective
USD500,000 from the Bill and Molly Ford
USD500,000 from Roots and Wings
USD138,000 from ADNIC
USD15,000 from DAF Giving 360

d. The BF earned USD859,144 from interest income in 2024 and 2025 July YTD. The income is kept in a dedicated BF bank account and will cover approved BF costs

Annex E: Potential partners

Multi-country	Malawi	Kenya	Rwanda	Tanzania
▪ CHAI ▪ Hatch Technologies ▪ Kenya Pediatric Association (KEPRECON) ▪ WHO ▪ COINN ▪ ECSACOMN ▪ SEED Global Health ▪ Society for African Neonatal Nurses ▪ University of Rwanda ▪ University of Zambia ▪ African Neonatal Association ▪ Rice University ▪ Ecumenical Pharmaceutical Network ▪ Axmed ▪ Health Information Systems Program (HISP)	▪ Pediatric and Child Health Association of Malawi ▪ Association of Obstetrician-Gynecologists of Malawi ▪ Association of Midwives in Malawi ▪ Kamuzu University of Health Sciences ▪ NEST Malawi (Flight) ▪ SEED Global Health ▪ CHAI Malawi	▪ Kenya Pediatric Association ▪ Kenya Obstetrical Gynaecological Society ▪ Midwives Association of Kenya ▪ University of Nairobi ▪ Moi University ▪ Kenyatta National Hospital ▪ Kenyatta University ▪ Getrude Children's Hospital ▪ Aga Khan University ▪ East African College of Paediatrics and Child Health ▪ NEST Kenya ▪ Jacaranda Health ▪ Amref Kenya ▪ Kisumu Medical Education Trust ▪ Hatch Technologies ▪ CHAI Kenya ▪ Touch Health ▪ Lwala ▪ PharmAccess Foundation ▪ KEMRI ▪ InSupply	▪ Rwanda Pediatric Association ▪ Rwanda Biomedical Centre ▪ University of Rwanda ▪ CHAI Rwanda ▪ Inshuti Mu Buzima ▪ University of Global Health Equity ▪ Imbunto Foundation ▪ MASS Design ▪ Rwanda Pediatrics Association ▪ Rwanda Society of Obstetricians & Gynecologists ▪ Rwanda Nursing and Midwifery Council	▪ Tanzania Pediatric Association ▪ Tanzania Association of Gynaecologists and Obstetricians ▪ Tanzania Midwifery Association ▪ Ifakara Health Institute ▪ Muhimbili University of Health and Allied Sciences ▪ Catholic University of Health and Allied Sciences ▪ Haydom Lutheran Hospital ▪ Aga Khan University ▪ InSupply ▪ Touch Health ▪ Amref ▪ Assist International ▪ CHAI Tanzania
Zimbabwe	**Ghana**	**Ethiopia**	**Uganda**	**Lesotho**
▪ Higherlife Foundation ▪ University of Zimbabwe ▪ OPHID ▪ Paediatric Association of Zimbabwe	▪ Paediatric Society of Ghana ▪ Christian Health Association of Ghana ▪ Kybele ▪ AYA Collective	▪ Ethiopia Federal Ministry of Health ▪ Ethiopia Regional Health Bureaus ▪ International Institute for Primary Health Care – Ethiopia ▪ Addis Ababa University ▪ Hawassa University ▪ Mekelle University ▪ Emory University Ethiopia office ▪ Ethiopian Pediatrics Society ▪ Hayat Biotech ▪ Institute for Healthcare Improvement ▪ Ethiopia Society of Obstetricians & Gynecologists ▪ Ethiopia Midwifery Association	▪ Uganda Paediatric Association ▪ The Registered Trustees of Uganda Episcopal Conference (UEC) ▪ Uganda Catholic Medical Bureau ▪ Adara Development Uganda ▪ Babies and Mothers Alive (BAMA)	▪ Higherlife Foundation ▪ CHAI